

Software Architecture Overview

Open Clinical Record

Open Clinical Record Internship Project

Draft — Discovery Phase

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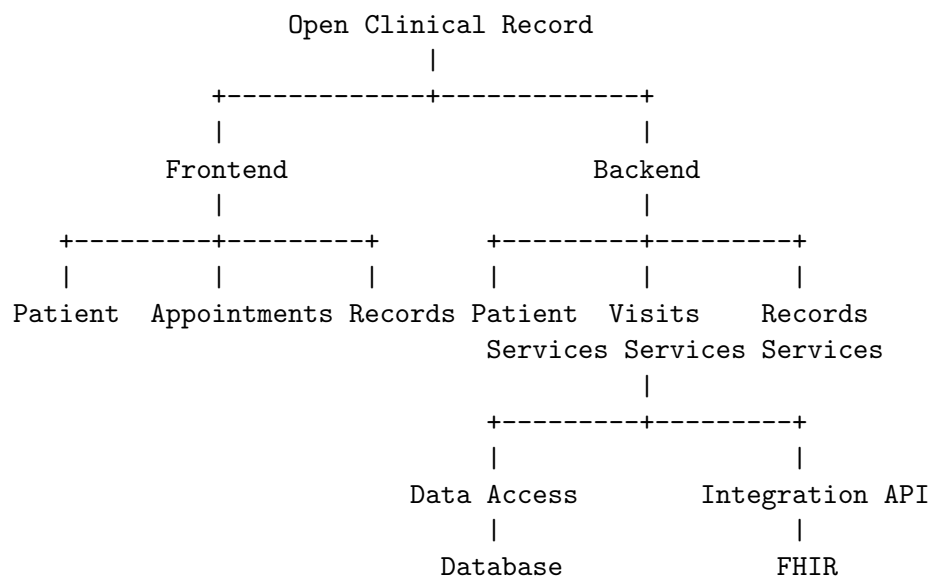
1 Status

This document records architectural direction during discovery. The final architecture will be based on validated requirements rather than being locked prematurely.

2 Architectural Intent

The system should be modular, maintainable, secure, and ready for API-based integration. The MVP should not require every future module to exist, but it should establish clean boundaries around the core clinical workflows.

3 Initial Logical View



4 Cross-Cutting Concerns

Authentication, authorization, validation, audit logging, error handling, configuration, and notification mechanisms should be treated as platform concerns rather than duplicated inside individual clinical features.

5 Expansion Strategy

Future modules such as laboratory, pharmacy, referrals, billing, and patient-facing applications should connect through explicit application/service boundaries. External interoperability will be investigated through standards such as HL7 FHIR where appropriate.

6 Architecture Decisions

Important decisions will be recorded separately as Architecture Decision Records (ADRs). Each ADR should explain the problem, the decision, alternatives considered, consequences, and the evidence behind the decision.

7 What Is Not Decided Yet

The final frontend/backend technology stack, deployment topology, database implementation details, service granularity, and exact FHIR integration scope remain open until requirements and research provide enough evidence.